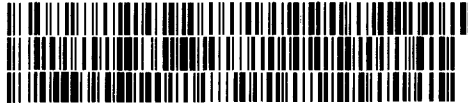


UUID:09A86147-EB23-489F-9849-9FDC91AC44A1
TCGA-3L-AA1B-01A-PR

Redacted



MRN: [REDACTED]
Patient: [REDACTED]
Admission Date:
Ordering Physician:

Sex/DOB: [REDACTED]
Discharge Date:

Female [REDACTED]

Pathology Addendum Report

Collected Date/Time:
Received Date/Time:

Accession Number: [REDACTED]

Addendum Report

**DNA MISMATCH REPAIR TESTING BY IMMUNOHISTOCHEMISTRY FOR THE ASSESSMENT OF
MICROSATELLITE INSTABILITY:**

INTERPRETATION:

- NO DEFICIENCY IDENTIFIED IN THE DNA MISMATCH REPAIR PROTEINS TESTED (INTACT DNA MISMATCH
REPAIR FUNCTION, MICROSATELLITE STABLE, MSS)

RESULTS:

MLH1: PRESENT
PMS2: PRESENT
MSH2: PRESENT
MSH6: PRESENT

CECE *ICD-O 3*
Adenocarcinoma NOS 8140/3
path
Adenocarcinoma in tubulovillous adenoma
8260/3
Site: Cecum C18.0
QW 2/24/14

NOTE:

Microsatellite instability is not detected in the carcinoma by immunohistochemistry; therefore, the likelihood that the carcinoma is due to sporadic or hereditary (i.e.: Lynch syndrome-associated) defective DNA mismatch repair is very low. However, immunohistochemistry may not detect rare polymorphisms or missense mutations in DNA mismatch repair proteins in up to 10% of cases. If strong clinical suspicion persists, additional testing is suggested.

Archived slides and paraffin blocks were reviewed for case [REDACTED] block A10 containing normal tissue and carcinoma was selected for MSI analysis. Immunohistochemical studies were performed on formalin fixed, paraffin-embedded tissue with adequate positive and negative control sections, including internal positive control tissue.

The performance characteristics of these antibodies were determined by the

They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high-complexity clinical laboratory testing.

REFERENCE:

Printed by:
Copied to:
Distribute to:

Page 1 of 5

Print Date/Time:

Patient Locations:

MRN:
Patient:

Sex/DOB: Female

Pathology Addendum Report

Collected Date/Time:
Received Date/Time:

Accession Number:

Geiersbach KB, Samowitz WS. Microsatellite instability and colorectal cancer. Arch Pathol Lab Med. 2011(Oct);135:1269-77

(Electronic signature)
Verified:

Surgical Pathology Report

Collected Date/Time:
Received Date/Time:

Accession Number:

Final Diagnosis

RIGHT COLON, RIGHT COLECTOMY:

- ADENOCARCINOMA, MODERATELY TO POORLY DIFFERENTIATED, INVASIVE INTO MUSCULARIS PROPRIA, ARISING AS MULTIPLE FOCI IN A TUBULOVILLOUS ADENOMA WITH HIGH GRADE DYSPLASIA AND MULTIPLE FOCI OF INTRAMUCOSAL CARCINOMA.
- NO TUMOR SEEN IN TWENTY-EIGHT LYMPH NODES.
- MARGINS OF RESECTION ARE FREE OF TUMOR.
- SEE SYNOPSIS REPORT AND NOTE.

(Electronic signature)
Verified:

Synoptic Report

SPECIMEN:

Terminal ileum
Cecum
Appendix
Ascending colon

PROCEDURE:

Right hemicolectomy

SPECIMEN LENGTH:

Specify: 23 cm

TUMOR SITE:

Cecum

TUMOR SIZE:

Greatest dimension: 4.5 cm
Additional dimensions: 4.5 x 1.6 cm

MACROSCOPIC TUMOR PERFORATION:

Not Identified

HISTOLOGIC TYPE:

MRN: [REDACTED]
Patient: [REDACTED]

Sex/DOB: Female [REDACTED]

Surgical Pathology Report

Collected Date/Time:
Received Date/Time:

Accession Number: [REDACTED]

Adenocarcinoma
HISTOLOGIC GRADE:
High-grade (poorly differentiated to undifferentiated)
HISTOLOGIC FEATURES SUGGESTIVE OF MICROSATELLITE INSTABILITY:
Intratumoral Lymphocytic Response (tumor-infiltrating lymphocytes)
None
Peritumor Lymphocytic Response (Crohn-like response)
Mild to moderate
Tumor Subtype and Differentiation
Mucinous tumor component (Percentage: 15 %)
MICROSCOPIC TUMOR EXTENSION:
Tumor invades muscularis propria
DISTAL MARGIN:
Uninvolved by invasive carcinoma
CIRCUMFERENTIAL (RADIAL) OR MESENTERIC MARGIN:
Uninvolved by invasive carcinoma
TREATMENT EFFECT:
No prior treatment
LYMPHATIC (SMALL VESSEL) INVASION (L):
Not identified
PERINEURAL INVASION:
Not identified
TUMOR DEPOSITS (discontinuous extramural extension):
Not identified
TYPE OF POLYP IN WHICH INVASIVE CARCINOMA AROSE:
Tubulovillous adenoma (with high grade dysplasia, multiple foci of intramucosal carcinoma and multifocal invasive carcinoma.)
PRIMARY TUMOR (pT):
pT2: Tumor invades muscularis propria
REGIONAL LYMPH NODES (pN):
pN0: No regional lymph node metastasis
Number examined: 28
Number involved: 0
DISTANT METASTASIS (pM):
Not applicable
ANCILLARY STUDIES:
Microsatellite instability (testing method: IHC, pending, results will be reported in an addendum)

NOTE: tumor size corresponds to the macroscopic measurement of the tubulovillous adenoma which harbours diffuse high grade dysplasia, multiple foci of intramucosal carcinoma and multiple foci of invasive carcinoma.
MSI-IHC will be reported in an addendum.

Source of Specimen

A Right Colon

MRN: [REDACTED]
Patient: [REDACTED]

Sex/DOB: Female [REDACTED]

Surgical Pathology Report

Collected Date/Time:
Received Date/Time:

Accession Number: [REDACTED]

Clinical Information

PRE-OP DIAGNOSIS: Right colon neoplasm
POST-OP DIAGNOSIS: Same
TYPE OF PROCEDURE: Hand assisted laparoscopic, right colectomy

Gross Description

The specimen is labeled "RIGHT COLON" and is received unfixed. It consists of terminal ileum measures 5 cm in length and 3.5 cm in greatest circumference, colon measures 18 cm in length and 8 cm in greatest circumference, appendix measures 8.5 cm in length and 0.7 cm in greatest diameter. The serosa is congested covered by mesocolonic fat measures 34 cm thickness. Upon opening, the lumen contains fecalith material. There is a friable, polypoid, centrally ulcerated pink-tan mass measuring 5.5 x 4.5 x 1.6 cm. The masses and the proximal colon, 2 cm from ileocecal valve, 7 cm from the proximal margin of resection and 11 cm from the distal margin of resection. The mass invades the muscularis propria, approximately 3 cm from the radial margin. The colon mucosa is congested with a focal black discolored area surrounding the mass. The ileocecal valve is unremarkable. The terminal ileum mucosa is congested with no lesions. There are few diverticulum with no gross evidence of perforation. The appendix serosa is congested. The wall is intact. The lumen is narrow, contains fecalith material. There are 20 lymph nodes ranging from 0.4-1.2 cm in maximum dimensions. Representative sections are submitted, the entire mass is submitted.

Section Key:

- A1-proximal margin
- A2-distal margin
- A3-mass with deepest margin
- A4-A21 mass
- A22-black mucosa surrounding mass
- A23-diverticulum
- A24-random sections of colon
- A25-random sections of terminal ileum and ileocecal valve
- A26-appendix
- A27-one lymph node bisected
- A28-one lymph node bisected
- A29-one lymph node bisected
- A30-one lymph node bisected
- A31-one lymph node bisected
- A32-one lymph node bisected
- A33-three lymph nodes
- A34-four lymph nodes
- A35- five lymph nodes
- A36-six lymph nodes
- A37-five lymph nodes

The specimen is in formalin for more than 6 hours and less than 72 hours
Time specimen was removed from the patient: .
Time specimen was placed in formalin : .

MRN: [REDACTED]
Patient: [REDACTED]

Sex/DOB: Female [REDACTED]

Surgical Pathology Report

Collected Date/Time:
Received Date/Time:

Accession Number: [REDACTED]

Ischemic time: 1 hour 31 minutes

Dictated by:

Special Stains / Slides
37 H&E

Tissue Code

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	BA	Date Reviewed: 12/11/13